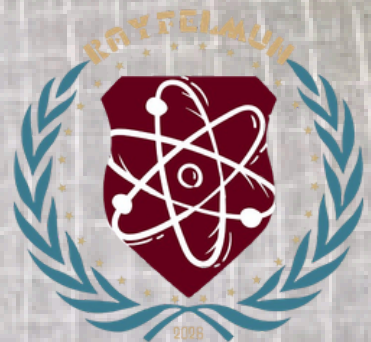


GA-3

SOCHUM

Rayfelman



Agenda Item: Preventing the Abuse of Euthanasia Practices While Respecting Human Dignity

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1. Letter From the Head of Academy

Dear Delegates;

With great enthusiasm, I welcome you all to the second edition of our school's Model United Nations conference! As the Head of the Academy, I am honored to witness the extraordinary talent and dedication each of you brings to this platform for dialogue and collaboration. This year, we are excited to present six dynamic committees, each tasked with addressing some of the most significant global issues.

In the Legal Committee, delegates will engage with one of the most complex and sensitive topics of our time: the legalization of abortion. The issue of abortion has been the subject of intense debate in different parts of the world for many years, shaped around fundamental concepts such as individual bodily autonomy, the right to life, public health, and social responsibility. States have adopted various policies on this issue in line with cultural and legal differences, leading to different practices internationally.

Considering different perspectives within a framework of mutual respect, examining the effects of existing practices, and developing proposals that can provide solutions to the problems facing the international community are the most important elements of our committee. During our discussions, we will comprehensively address issues such as human rights, women's health, societal impacts, and legal regulations.

Thank you for being part of this journey. I wish you thoughtful discussions, meaningful connections, and an inspiring MUN experience. If you have any questions, my door is always open.

Sincere Regards,

Defne YAMAN *Head of*
Academy

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2-LETTER FROM THE UNDER SECRETARY GENERAL OF SOCHUM

I take immense pride in welcoming you to RAYFELMUN's second edition, as well as the Social,

Humanitarian, and Cultural Committee. As the Third Committee in the United Nations, SOCHUM is at the forefront in the protection of human dignity across the globe. From securing the basic rights and freedoms of individuals to dealing with some of the most complex issues faced by humanity, such a committee requires more than knowledge; it requires empathy, diplomacy, and logic. In this session, we will deliberate on a deeply complex agenda: *Preventing the Abuse of Euthanasia Practices While Respecting Human Dignity*. This topic sits at the crossroads of bioethics, law, and human rights. It challenges us to find the delicate equilibrium between protecting vulnerable populations from coercion and upholding an individual's right to a dignified existence. Your task is to craft structured, ethical solutions that honor both the sanctity of life and the autonomy of the person.

My advice is to enter these deliberations with an open mind and to focus on constructive criticism and cooperation. These resolutions you will create are not just classroom exercises, but rather represent the intricacies of our world today. Whether you are a seasoned or a first-time delegate, your perspective is vital to our collective success. I look forward to witnessing your insightful debates, your commitment to diplomacy, and your innovative approaches to these global challenges.

In case you need any clarifications during the conference, please do not hesitate to reach out.

Best regards,

Ekin Sezer

Under Secretary General

3 - Introduction to the Committee

a. What is Sochum?

SOCHUM (Social, Humanitarian, and Cultural Committee) is the Third Committee of the United Nations General Assembly. It is involved in the discussion of many different aspects relating to human rights, social progress, and humanitarian affairs. These include the protection of vulnerable communities and individuals, promotion of basic human freedoms, and promotion of equality and dignity for all individuals. SOCHUM is one of the most important committees in which nations discuss difficult problems of modern times, including discrimination, refugee crises, human trafficking, and moral dilemmas that are associated with human rights. The recommendations developed by the committee are based on the collaboration between many UN agencies and other international organizations and can be seen as effective tools to address many pressing social issues around the world. In SOCHUM, diplomacy and cooperation are crucial components.

4 - INTRODUCTION TO THE AGENDA ITEM

a. Understanding the Agenda: Key Terms and Definitions

Vulnerable Populations: Individuals or groups of people who are more vulnerable to being coerced, pressurized, or restricted when it comes to health access and, hence, may be more vulnerable to euthanasia abuses.

Coercion: Coercion can be defined as the act of using threats or manipulation to push someone into making a certain decision against their wishes.

Human Dignity: Human dignity refers to the inherent dignity that everyone possesses, including the need to respect, protect, and uphold a person's integrity.

Right to Life: It refers to the basic human right of all people to live and be allowed to live without having their lives taken away from them unnecessarily.

Right to Die: It is the right of an individual to determine their time of death, especially where there is no hope of recovery.

Slippery Slope: The fear that permitting a particular act (euthanasia under certain conditions) will cause more dangerous acts to happen later.

Medical Ethics: Standards of conduct in medicine that are intended to assist the physician to make proper choices concerning patient treatment, particularly where the patient's life may be at stake.

Depression: An illness where an individual experiences consistent periods of sadness, lack of enthusiasm or interest, low energy levels, and impaired judgment, thereby affecting one's sense of reality regarding their life and future.

Capacity: It is the capacity of an individual, legally and cognitively, to comprehend information and make sound choices concerning their medical decisions. This concept is important in euthanasia since full consent is vital.

Autonomy: Autonomy is the idea that an individual should have the freedom and capacity to make sound judgments on matters about one's life, body, and future.

5 - Background of the Agenda Item

a. Euthanasia

The concept of euthanasia, otherwise referred to as "mercy killing," is believed by many people to be one of the most controversial topics currently facing the discipline of bioethics. Euthanasia refers to the intentional cessation of a person's life in order to relieve extreme pain and suffering. As a result of advancements in technology that allow healthcare professionals to extend individuals' lifespans far past their due date, the debate regarding "the right to die" has intensified.

Euthanasia can be broadly classified into three categories based on the patient's consent. First, there is voluntary euthanasia, whereby an individual, who is mentally sound and competent, requests their own death. Second, there is non-voluntary euthanasia whereby individuals who cannot make decisions about their deaths, for example, patients in a coma, have their relatives decide on their behalf. Third, involuntary euthanasia. The practice of involuntary euthanasia involves causing the death of an individual against their will, often to relieve severe distress caused by sickness or other ailments. Involuntary euthanasia is very controversial because it contravenes human rights. Additionally, there are active euthanasia and passive euthanasia. Active euthanasia entails the injection of lethal drugs, while passive euthanasia involves the withdrawal of treatments that keep patients alive, such as respirators.

The legal aspects of the world are very confusing because countries espoused different ideological positions on the issue of euthanasia. The Dutch, Belgians, and Canadians have adopted very strict regulations on active euthanasia. However, in most other countries, including some American states, physician-assisted suicide and passive euthanasia are allowed. It could be noted that in such countries, the legal system is designed in a way that the process would be "sanitized," meaning that the patient's wish is respected, but in a way that ensures that the act will not happen at the hands of the doctor.

The ethical reasons behind the justification of euthanasia lie in the concept of self-determination. It is argued that people have the right to live in dignity, which means that they also deserve a dignified death. When an individual suffers from a terminal disease, and his or her condition cannot be alleviated by palliative care, he or she should have the option of a peaceful death without suffering.

On the other hand, critics have several reasons for being concerned about the so-called "slippery slope." First of all, legalization of euthanasia would mean that people might start considering life less valuable, making elderly people, disabled people, and mentally ill people feel like there was a "duty to die," to prevent being a burden to their relatives both emotionally and financially. Moreover, in accordance with most religions and philosophies, life is regarded as something inherently valuable that cannot be ended through artificial means, no matter what situation one is in. Another issue that raises debate about euthanasia is that it might contradict the Hippocratic Oath, which is the standard of ethics that medical doctors swear to uphold, and focuses on the preservation of life and protection of patients from harm. Some people say that the process of helping patients end their lives contradicts the principles of ethics for doctors, but some believe it to be a compassionate choice as well.

b. Types of Euthanasia

i. Active Euthanasia

The topic of active euthanasia, which entails the practice of intentionally ending the life of a person to alleviate terminal agony, is arguably one of the most contentious aspects in contemporary bioethics. On one hand, recent medical breakthroughs have provided humans with more means to prolong lives than ever before. On the other hand, these advancements have produced conditions where human lives can only be preserved at the expense of immense pain and degradation. Consequently, the issue of active euthanasia touches upon a conflict between two major bioethical values – personal autonomy and the preservation of life. According to its proponents, euthanasia should not be viewed as an intentional ending of a life but as an option for having a “much better death.”

There exist two main types of euthanasia: passive and active euthanasia. The former includes the termination of the supply of any medication necessary to maintain a patient's life. Active euthanasia, also known as aggressive euthanasia, refers to the act of actively bringing death into effect through lethal substances.

The first fundamental difference between voluntary and non-voluntary euthanasia should be pointed out in the context of active euthanasia. If voluntary euthanasia involves the act of ending life based on the patient's informed and consensual decision, non-voluntary euthanasia entails making that choice for a patient who is unconscious or otherwise unable to voice their opinion. While it might be assumed that the decision would then reflect the patient's best interests, the fact remains that

the actual opinions of a non-responsive individual cannot always be ascertained. At the same time, involuntary euthanasia is characterized by killing an individual against their will, which is considered an extremely severe crime comparable to murder and strictly prohibited by the law.

It might be argued that active euthanasia entails several serious and highly questionable implications despite all attempts to prove its feasibility. The main challenge associated with it is related to the likelihood of abuse and coercion. For example, the elderly, individuals with disabilities, and financially struggling persons may experience implicit pressure to opt for active euthanasia because this act would not reflect their actual desires but rather impose their perceived responsibility to ease other people's suffering. This undermines the very principle of voluntary choice that advocates emphasize.

Connected to this issue is the difficulty in determining genuine consent and mental capacity. Psychological issues like depression, fear, or mental disorders can greatly affect how patients make decisions. Evaluating whether the patient has made an informed, rational, and consistent choice when asking for euthanasia is already complicated by nature; making a mistake would result in irrevocable consequences.

The slippery slope theory adds another layer to the debate on euthanasia. Some have argued that the act of legalizing the procedure for specific cases will eventually evolve into something else, where it would even cover illnesses other than terminal diseases, people who cannot give their consent, and even minors.

From the perspective of medicine, active euthanasia goes against the very basics of ethical medicine, namely the principle of “first do no harm.” The deliberate taking of lives is perceived as contradicting the essence of what doctors stand for, creating serious issues relating to the trust gap between patients and physicians.

Legal considerations present yet another aspect that makes the task even more challenging. Developing measures that would be effective enough and enforced regardless of circumstances is quite difficult. In addition, the differing approaches of various nations to the delicate question of weighing the right to live against the right to die create legal discrepancies and loopholes.

Further, social and philosophical consequences should be discussed. For instance, proponents of active euthanasia suggest that it can have subtle effects on the valuation of human life, especially regarding chronically ill and disabled people.

Overall, it can be stated that these issues are responsible for causing active euthanasia to be one of the most controversial global topics. Indeed, active euthanasia is not just a matter of medicine or law; rather, it is an ethical question.

ii. Passive Euthanasia

Passive euthanasia is defined as the act of letting a patient die naturally by not providing medical treatment that sustains life. Passive euthanasia differs from active euthanasia in that the former does not kill a person, but rather allows an illness or condition to run its course, while the latter involves an action meant to hasten death.

Passive euthanasia often arises when a treatment merely prolongs a dying process without prospects for improvement. Examples include disconnecting machines that keep patients breathing, stopping artificial feeding, and not administering *CPR (Do Not Resuscitate orders)*.

The basic principle behind passive euthanasia is the recognition of the autonomy of the patient. A competent individual has the right to reject any form of treatment, even if their decision could lead to their own death. Decisions regarding patients who cannot make their own decisions usually depend on their advanced directives or living wills, or on the advice of their families and physicians acting in their best interest.

Unlike active euthanasia, which involves taking affirmative action to bring about a patient's death, passive euthanasia is generally more accepted from a legal and moral perspective around the globe.

Nevertheless, there have been some debates regarding the use of passive euthanasia, with some people thinking that the moral difference between "letting die" and "killing" may, at times, become confusing, as the results would always be death.

Moreover, the authenticity of the person's decision, especially during non-voluntary passive euthanasia, has come under criticism as well.

Regardless of all these discussions, passive euthanasia continues to play an important role in end-of-life issues.

iii. Voluntary Euthanasia

Voluntary euthanasia is the practice in which a mentally competent individual freely and explicitly consents to end their own life to escape unbearable suffering, most commonly caused by a terminal or incurable illness. The key defining element is informed consent, meaning the patient fully understands their medical condition, prognosis, available alternatives, and consequences of the decision, and chooses euthanasia without coercion, pressure, or impaired judgment.

Voluntary active euthanasia involves a direct medical action to end life, typically through the administration of a lethal substance by a healthcare professional. Voluntary passive euthanasia, on the other hand, occurs when a patient chooses to refuse or withdraw life-sustaining treatment, such as ventilation, medication, or artificial feeding, allowing the underlying condition to lead to a natural death.

At its heart, voluntary euthanasia revolves around the concept of autonomy, which refers to the rights of an individual to make decisions about his or her body and personal life. Voluntary euthanasia proponents feel that such decisions must include what happens to one at the end of his or her life in cases where one cannot avoid suffering and the inability to regain independence in life.

However, the practice remains highly controversial due to several ethical and practical concerns. One major issue is the reliability of consent. Patients may be influenced by depression, emotional distress, pain, or feelings of being a burden to others, raising doubts about whether their decision is fully free and stable over time. Ensuring that consent is truly informed and consistent is therefore a major challenge.

The question of the adequacy of legal and medical precautions in this procedure is another point for consideration. While in cases where voluntary euthanasia is legalized, there are tight rules such as several medical opinions, psychological examination, and a terminal condition, among others, the problem is that errors cannot be ruled out even under these precautions.

The issue of whether or not voluntary euthanasia is an acceptable action also opens up a range of ethical dilemmas regarding the medical profession. In this regard, the act contradicts the

Hippocratic principle of "Do No Harm" since, in this case, instead of prolonging people's lives, the opposite action is taken.

Moreover, it should be noted that voluntary euthanasia is only allowed in a limited number of states; therefore, from a legal perspective, in most countries, such actions are prohibited and strictly punished.

iv. Non-Voluntary Euthanasia

Non-voluntary euthanasia means ending someone's life when they can't consent because they're unconscious, have severe mental limitations, or can't communicate. Usually, family members, guardians, or doctors make this decision, thinking it's in the patient's best interest. It's generally used when there's no hope of getting better and the person is enduring lots of suffering, whether that's physical pain or emotional distress.

Non-voluntary euthanasia is super controversial because of the lack of explicit consent. This absence makes it hard to know what the patient actually wants. Decisions are tough, too, since there's always a risk of misunderstanding or bias. It gets even trickier with emotions or money stirring things up. So, people get worried about acting ethically without that vital patient input.

A big debate revolves around who should make those permanent calls. Giving that power to family members or doctors brings up issues about accountability. Plus,

there's the problem that stand-ins might not fully know the patient's values, especially for critical choices

Safeguards and oversight complications come up, too. Making sure decisions are ethical and there's no misuse is tough. This is particularly difficult when rules are murky or inconsistently applied. Also, the chance of errors, like a wrong diagnosis or judgment on quality of life, makes these decisions even trickier since they can't be undone.

Finally, non-voluntary euthanasia sparks wider societal worries. It's about valuing all lives, especially those of the most vulnerable. Some fear it could slowly change how we view the lives of people with serious disabilities or who are minimally conscious, maybe seeing their lives as less worth protecting.

v. Involuntary Euthanasia

Involuntary euthanasia is when someone ends a person's life without that person's consent, even though the individual can make informed choices. This practice is pretty much the most criticized and banned form of euthanasia out there. It differs from others because it disregards what the person wants, clearly violating their wishes and major ethical guidelines.

At heart, this kind of euthanasia violates a person's autonomy – their ability to choose what happens to their body and life. When healthcare providers go against someone's no to treatment, they take away that person's fundamental right to self-determination. It disrespects the individual's dignity and rights. That's why involuntary euthanasia is seen more as a wrongful act than anything else, definitely not a legit medical procedure.

The big worry with involuntary euthanasia is how it could be abused. Giving anyone (the doctors, the government, or caretakers) the power to end someone's life without their OK sets up a dangerous situation. Decisions could easily be swayed by personal biases or shady outside pressures.

People who are already vulnerable, like the elderly, folks with disabilities, or those struggling economically, are especially at risk. Involuntary euthanasia also sparks major problems about who's responsible and how to keep watch. When you remove a person's consent, there's no way to guarantee their best interests are being considered.

It leaves room for mistakes, self-interest, or worse. If we don't have ironclad laws in place, these practices could totally wreck someone's rights, and there'd be nothing to stop it.

Legally speaking, involuntary euthanasia is nearly always banned and usually considered homicide or murder. Legal systems protect the right to life vigorously,

viewing acts that end a life without consent as serious crimes. This global agreement highlights just how much people think the practice goes against basic ethics.

If we accepted or made involuntary euthanasia common, trust in healthcare would nosedive. Patients depend on doctors to safeguard life, honor their preferences, and provide care that's in their best interest. Even the hint that doctors might end someone's life without permission could destroy faith in hospitals and other health services, keep sick folks from getting help, and tarnish the reputation of the entire medical field.

Plus, there are deep social and ideological concerns. Permitting involuntary euthanasia might suggest some lives are deemed worth less by others, leaving out the individuals themselves, who should decide for themselves. This could contribute to discrimination and reinforce harmful attitudes toward those who are ill, disabled, or dependent. It challenges the fundamental principle that all human lives possess equal worth and deserve protection.

c. Controversy Surrounding Euthanasia

Euthanasia is highly controversial because it sits at the intersection of ethics, law, medicine, and personal belief, raising fundamental questions about life, death, and human dignity. While some view it as an act of compassion that relieves unbearable suffering, others see it as morally wrong and potentially dangerous. This deep divide is what makes the issue so complex and widely debated.

One of the main reasons for controversy is the conflict between the right to life and the right to die. Supporters argue that individuals should have autonomy over their own bodies and the freedom to choose a dignified death, especially in cases of severe illness. Opponents, however, believe that life is inherently valuable and should be protected under all circumstances, regardless of suffering. This creates a fundamental ethical disagreement that is difficult to reconcile.

Another major issue is the risk of abuse and mistakes. Concerns such as misdiagnosis, premature decisions, and weak legal safeguards raise the possibility that euthanasia could be carried out in cases where it is not truly justified. Because the decision is irreversible, even a small margin of error becomes extremely serious. Critics worry that vulnerable groups such as the elderly, disabled, or mentally ill may be particularly at risk.

The role of mental health also adds to the controversy. Conditions like depression can influence a person's desire to die, making it difficult to determine whether a request for euthanasia is truly rational and stable. This challenges the idea that euthanasia is always a fully autonomous decision and raises questions about how to assess consent properly.

Additionally, the “slippery slope” argument suggests that once euthanasia is legalized under strict conditions, those boundaries may gradually expand. What begins as an option for terminal illness could extend to less severe cases, potentially weakening protections over time. This possibility makes many people cautious about supporting legalization.

Religious and cultural beliefs further contribute to the debate. In many traditions, life is considered sacred, and intentionally ending it is seen as morally unacceptable. These beliefs strongly influence public opinion and legal systems in different countries, leading to varying approaches to euthanasia around the world.

Finally, there is concern about the potential devaluation of life. Some argue that allowing euthanasia may send a message that certain lives, particularly those affected by illness or disability, are less worth living. This could shape societal attitudes and impact how vulnerable individuals are treated and perceived.

d. Personal Autonomy

Personal autonomy is one of the central arguments in the debate on euthanasia and refers to an individual’s right to make independent decisions about their own life and body. In the context of end-of-life care, autonomy is often used to support the idea that a person should have control over the timing and manner of their death, especially when facing unbearable suffering or a terminal illness.

Supporters of euthanasia argue that autonomy is a core principle of human dignity. From this perspective, forcing a person to continue living in extreme pain or with no hope of recovery can be seen as a violation of their self-determination. They claim that if individuals are competent and fully informed, they should have the right to choose euthanasia as a way to avoid prolonged suffering. In this view, respecting autonomy means respecting a person’s deeply personal judgment about what makes their life meaningful or intolerable.

However, the application of autonomy in euthanasia is not straightforward. One major concern is whether decisions are always truly free and fully informed. Patients requesting euthanasia may be influenced by depression, anxiety, pain, or feelings of being a burden on their family. In such cases, their decision may not reflect stable, long-term autonomy but rather temporary emotional distress. This raises questions about whether true autonomous consent can always be guaranteed.

Another issue is that autonomy in healthcare is not absolute. Medical ethics often balance autonomy with other principles such as *non-maleficence* (do no harm) and *beneficence* (acting in the patient’s best interest). Doctors may refuse certain requests if they believe the decision is not in the patient’s best interest or if the patient’s judgment is impaired. In euthanasia debates, this creates tension between respecting individual choice and protecting individuals from irreversible harm.

Additionally, external pressures can limit autonomy. Vulnerable individuals, such as elderly patients or those with disabilities, may feel indirect pressure to choose euthanasia due to financial burdens, lack of care, or emotional stress on their families. Even if consent is formally given, such pressures can undermine the authenticity of the decision. **6- RISKS AND ABUSE**

The risks and potential abuse of euthanasia form a central pillar of the global ethical debate surrounding end-of-life care. Although euthanasia is often presented as a compassionate response to alleviate unbearable suffering, it also introduces a range of serious concerns that cannot be overlooked. One of the primary risks is the possibility of coercion, where patients, especially those who are elderly, disabled, or financially dependent, may feel subtle pressure to choose death to avoid being a burden on others. Additionally, determining whether a patient's consent is truly informed and voluntary is highly complex, as factors such as depression, fear, pain, or inadequate medical and psychological support can significantly influence decision-making. There are also concerns about medical fallibility, including misdiagnosis or inaccurate prognoses, which could result in premature and irreversible decisions. Furthermore, critics warn of a potential "slippery slope," where the legalization of euthanasia under strict conditions may gradually expand to include broader and more ethically questionable cases, weakening safeguards over time. These issues, combined with challenges in legal oversight and enforcement, highlight the difficulty of ensuring that euthanasia remains ethical, controlled, and free from abuse, making it one of the most complex and sensitive topics in modern bioethics.

a. Pressure on elderly/disabled people

One of the most concerning issues in the euthanasia debate is the pressure, both subtle and systemic, placed on elderly and disabled individuals, which can significantly influence their end-of-life decisions. Even when no direct coercion exists, many individuals in these groups may develop a strong internalized belief that they are a burden on their families, caregivers, or society at large. This feeling is often reinforced by emotional dependence, declining physical ability, and the need

for continuous medical or financial support, which can create a sense of guilt or responsibility that pushes them toward considering euthanasia as a way to "relieve" others rather than a truly free personal choice.

This pressure is further amplified by societal values that prioritize independence, productivity, and physical ability, sometimes leading to the implicit idea that lives affected by disability, chronic illness, or advanced age are less meaningful or less dignified. Even without explicit statements, these cultural attitudes can shape how individuals perceive their own worth, making them more vulnerable to the belief that their continued existence imposes a burden. In such environments, the decision to

pursue euthanasia may not stem purely from personal desire to end suffering, but from an underlying sense of obligation or reduced self-worth.

Another important factor is the unequal access to healthcare and social support systems. In many cases, inadequate palliative care, insufficient psychological support, or financial limitations can leave elderly and disabled individuals feeling isolated or without viable alternatives for managing their suffering. When proper care is unavailable or difficult to access, euthanasia may not appear as one option among many, but as the only practical solution to avoid prolonged pain or dependency. This raises serious concerns about whether the choice is truly voluntary or shaped by external constraints.

Additionally, emotional states such as depression, anxiety, loneliness, and fear of future deterioration can heavily influence decision-making in vulnerable individuals. These conditions may distort perception of quality of life, leading to choices that might differ significantly if adequate emotional and psychological support were provided. In this context, consent becomes difficult to evaluate, as it is unclear whether the desire for euthanasia reflects a stable, autonomous decision or a response to temporary or treatable distress.

b. Misdiagnosis and Premature Decisions

Misdiagnosis and premature decision-making are among the most serious risks in the practice of euthanasia, as they can lead to irreversible outcomes based on flawed or incomplete information. Because euthanasia involves intentionally ending a life to relieve suffering, it requires a level of certainty and careful judgment that medicine does not always guarantee. Even with advanced technology and expertise, diagnoses can be incorrect, and prognoses can change over time, making caution essential in all cases.

Misdiagnosis is particularly dangerous in this context. A patient may be incorrectly identified as having a terminal or untreatable condition when, in reality, their illness is manageable, or new treatments may become available. Medical history has shown that some patients outlive initial expectations or even recover after being given a poor prognosis. If euthanasia is carried out based on such errors, it eliminates any possibility of correction or recovery. This raises a critical ethical concern: decisions about life and death may be made under uncertainty, where absolute accuracy is not always possible.

Premature decisions also pose a significant challenge. Patients facing severe illness often experience emotional distress, fear, or depression, which can influence their judgment. A request for euthanasia made during a moment of despair may not reflect a stable, long-term wish. Over time, with proper palliative care, psychological support, and adaptation to their condition, many patients may regain a sense of

meaning or improved quality of life. Acting too quickly removes the opportunity for these changes to occur.

Furthermore, external pressures can contribute to premature decision-making. Patients may feel like a burden on their families or worry about financial and emotional costs, which can subtly shape

their choices. In such situations, the decision may not be entirely autonomous. Without thorough evaluation and sufficient time for reflection, there is a risk that euthanasia is chosen for reasons that could have been addressed through better support systems.

To reduce these risks, strong safeguards are necessary. These include multiple independent medical opinions, accurate and repeated diagnostic assessments, psychological evaluations, and mandatory waiting periods. Ensuring access to high-quality palliative care is also crucial, as it can alleviate suffering without resorting to life-ending measures.

c. Mental Health Issues

Mental health issues play a critical role in the debate over euthanasia, particularly because they can affect a person's judgment, decision-making capacity, and perception of their own quality of life. While euthanasia is often framed as a rational and autonomous choice, this assumption becomes more complicated when psychological conditions such as depression, anxiety, or trauma are involved. These conditions can distort thinking, making individuals more likely to view their situation as hopeless or unbearable, even when effective treatment and support options exist.

One of the main concerns is that mental health disorders, especially depression, can directly influence a person's desire to die. Depression is strongly associated with feelings of worthlessness, loss of meaning, and persistent hopelessness. In such a state, a request for euthanasia may not reflect a clear, stable, and informed decision, but rather a symptom of the illness itself. Importantly, many mental health conditions are treatable, and individuals who once expressed a desire to die may later recover and find value in continuing life. This raises serious ethical questions about whether it is ever appropriate to grant euthanasia when mental health is a significant factor.

Another issue is the difficulty of accurately assessing mental capacity in patients with psychological conditions. Determining whether a person is making a truly autonomous decision requires evaluating their ability to understand their situation, consider alternatives, and make a reasoned choice. However, mental health disorders can impair these abilities in subtle ways, making assessments complex and sometimes unreliable. As a result, there is a risk that individuals who are not fully capable of informed consent may still be approved for euthanasia.

Additionally, social and emotional factors often intersect with mental health. Feelings of isolation, being a burden, or lack of support can intensify psychological distress and influence decisionmaking. In such cases, the desire for euthanasia may stem less from physical suffering and more from unmet emotional or social needs. Addressing these underlying issues through therapy, community support, and improved mental health care could significantly reduce such requests.

To address these concerns, strict safeguards are essential. Comprehensive psychological evaluations, involvement of mental health professionals, and mandatory waiting periods can help ensure that decisions are not made during temporary crises. Expanding access to mental health treatment and support systems is equally important, as it provides alternatives to euthanasia and helps individuals cope with their suffering in non-lethal ways.

d. Weak Legal Frameworks

Weak legal safeguards are one of the most significant risks in the practice of euthanasia, as they can open the door to misuse, abuse, and irreversible mistakes. Because euthanasia involves intentionally ending a human life, it requires strict legal frameworks to ensure that decisions are voluntary, wellinformed, and free from external pressure. When these safeguards are insufficient, the system becomes vulnerable to errors and exploitation.

One major concern is the lack of clear and consistent criteria for eligibility. If laws are too vague about what constitutes “unbearable suffering” or a “terminal illness,” it leaves room for subjective interpretation by medical professionals. This can result in inconsistent decisions, where similar cases are treated differently, or where euthanasia is granted in situations that were not originally intended by the law. Over time, this may lead to a gradual expansion of eligibility, often referred to as the “slippery slope,” where safeguards weaken, and more cases are approved under broader conditions.

Another issue is inadequate oversight and accountability. In some systems, euthanasia cases are reviewed only after the procedure has been carried out, limiting the ability to prevent wrongful decisions. Without strong monitoring bodies, transparent reporting, and independent review processes, it becomes difficult to detect patterns of abuse or ensure that legal requirements are consistently followed. This lack of oversight can reduce trust in both the healthcare system and the law itself.

Weak safeguards also increase the risk of coercion or subtle pressure on vulnerable individuals. Patients who are elderly, disabled, or financially dependent may feel influenced, directly or indirectly, to choose euthanasia. If legal frameworks do not require thorough assessments of voluntariness or fail to protect against external

influence, decisions may not truly reflect the patient's free will. This undermines the ethical principle of autonomy, which is often used to justify euthanasia.

Additionally, insufficient requirements for psychological evaluation can be dangerous. Without mandatory mental health assessments, patients suffering from depression or emotional distress may be approved for euthanasia without fully addressing treatable conditions. This links closely to the broader issue of ensuring that all alternatives, such as palliative care and psychological support, have been properly explored before proceeding.

To reduce these risks, strong and clearly defined legal safeguards are essential. These include precise eligibility criteria, multiple independent medical opinions, mandatory psychological evaluations, waiting periods, and rigorous oversight by independent authorities. Transparent documentation and regular audits can further ensure accountability and public trust. e. Slippery Slope

The "slippery slope" argument is one of the most frequently discussed concerns in debates about euthanasia. It refers to the idea that once euthanasia is legalized under strict and limited conditions, those boundaries may gradually expand over time, leading to broader and potentially less justifiable applications. What begins as an option for terminally ill patients experiencing unbearable physical suffering could, critics argue, extend to non-terminal conditions, psychological suffering, or even cases where consent is unclear.

One key aspect of this concern is the gradual expansion of eligibility criteria. In some countries where euthanasia or assisted dying is legal, the law has evolved to include a wider range of cases than originally intended. For example, initial policies may have focused only on patients with terminal illnesses, but over time, eligibility can expand to include chronic illnesses, disabilities, or mental health conditions. This raises ethical questions about where the line should be drawn and whether it can be effectively maintained once crossed.

Another issue is the normalization of euthanasia within healthcare systems and society. As the practice becomes more accepted, both doctors and patients may begin to view it as a standard option rather than a last resort. This shift in perspective can subtly influence decision-making, especially for vulnerable individuals who may already feel like a burden. Over time, societal attitudes toward life, suffering, and care may change in ways that reduce the emphasis on preserving life at all costs.

The slippery slope argument also highlights risks related to weakened safeguards. Even if strict regulations are initially in place, enforcement may become more lenient, or exceptions may be made in complex cases. This can lead to inconsistencies and a gradual erosion of the protections designed to prevent misuse. In such situations, cases that would have previously been rejected might begin to be approved, further expanding the scope of euthanasia.

Supporters of euthanasia often challenge the slippery slope argument, stating that strong legal safeguards and oversight can prevent such expansion. However, critics maintain that no system is immune to change over time, especially when influenced by social, economic, and medical pressures.

f. Devaluation of Life

The concept of the “devaluation of life” is a central ethical concern in debates about euthanasia. It refers to the idea that allowing the intentional ending of life under certain conditions may gradually reduce the perceived value and dignity of human life, especially for those who are vulnerable, such as the elderly, disabled, or chronically ill. While euthanasia is often justified as a compassionate response to suffering, critics argue that it can unintentionally send a message that some lives are less worth living than others.

One key issue is how society defines “quality of life.” When euthanasia is permitted based on judgments about suffering or reduced quality of life, it introduces a subjective standard that may vary between individuals, cultures, and medical professionals. This can lead to the perception that people who are dependent, disabled, or seriously ill are living lives of lesser value. Over time, such attitudes may influence both public opinion and healthcare practices, subtly reinforcing discrimination or bias against vulnerable groups.

Another concern is the impact on how patients view themselves. Individuals facing illness or disability may internalize societal attitudes and begin to feel that they are a burden to others or that their lives lack meaning. In such cases, a request for euthanasia may not stem purely from personal autonomy, but from feelings shaped by external perceptions and social pressure. This challenges the ethical foundation of euthanasia as a fully voluntary and independent choice.

The devaluation of life can also affect the medical profession. Traditionally, healthcare is guided by the principle of preserving life and providing care, even in difficult circumstances. If euthanasia becomes more normalized, it may shift the role of medical professionals from solely healing and supporting patients to also participating in ending life. This change can alter the ethical framework of medicine and influence how care is delivered, particularly for those with long-term or complex conditions.

Furthermore, there is a broader societal implication. If euthanasia becomes widely accepted, it may reduce the urgency to invest in palliative care, mental health services, and support systems for those in need. Instead of improving the conditions that cause suffering, there is a risk that ending life becomes seen as a solution. This could disproportionately affect individuals who lack access to adequate healthcare or social support.

7. LEGAL STATUS AROUND THE WORLD

a. Countries Where Euthanasia Is Legal

Netherlands – One of the first countries to legalize euthanasia (2002). Allowed under strict conditions, such as unbearable suffering and a voluntary, well-considered request.

Belgium – Legal since 2002. Includes adults and, in rare cases, minors under strict medical and psychological evaluation.

Luxembourg – Legal since 2009, with strict eligibility requirements similar to Belgium and the Netherlands.

Spain – Legal since 2021. Allows euthanasia and assisted suicide for serious, incurable conditions causing unbearable suffering.

Canada – Medical Assistance in Dying (MAID) is legal. Initially limited to terminal illness, but eligibility has expanded in recent years.

Colombia – Euthanasia has been decriminalized and regulated through court decisions and medical guidelines.

New Zealand – Legal since 2021 for terminally ill adults meeting strict criteria.

Austria – Assisted suicide is legal under strict conditions (since 2022), though euthanasia itself is not permitted.

Germany – Assisted suicide is permitted following court rulings, but active euthanasia remains illegal.

United States – Assisted dying is legal in some states (e.g., Oregon, California, Washington), but euthanasia is not generally legal nationwide.

Australia – Voluntary assisted dying is legal in several states (such as Victoria and New South Wales), under strict eligibility rules.

b. Countries Where Euthanasia Is Illegal

Türkiye – Euthanasia is strictly illegal under criminal law. Any intentional act of ending a patient's life, even with consent, is considered homicide. The focus is on palliative care and protecting the sanctity of life.

China – Euthanasia is not permitted under any legal framework. Medical law prioritizes preserving life, and end-of-life decisions are limited to treatment refusal in specific cases.

Japan – Active euthanasia is illegal. However, passive withdrawal of treatment may be allowed under strict medical and legal guidelines in rare end-of-life situations.

India – Active euthanasia is illegal, but passive euthanasia is permitted under strict conditions approved by the Supreme Court, such as withdrawal of life support in terminal cases.

South Korea – Euthanasia is illegal in all forms. The legal system emphasizes life preservation, though end-of-life care policies are slowly developing.

Saudi Arabia – Euthanasia is strictly prohibited due to strong religious and legal principles. Life is considered sacred, and intentionally ending it is treated as a serious crime.

Iran – Euthanasia is illegal under Islamic law and national legislation. Any act of assisted death is considered morally and legally unacceptable.

France – Euthanasia is illegal, but deep sedation and end-of-life palliative practices are allowed under strict medical supervision.

Italy – Euthanasia is illegal, although court rulings have allowed assisted dying in very limited exceptional cases. The issue remains highly debated.

Poland – Euthanasia is strictly illegal under criminal law, with strong protections for the right to life.

Russia – Euthanasia is completely prohibited. Medical professionals are legally required to preserve life at all costs.

Brazil – Euthanasia is illegal, though patients may refuse treatment in certain medical circumstances.

Nigeria – Euthanasia is illegal and treated as equivalent to homicide, with strong legal and cultural opposition.

8. INTERNATIONAL LEGAL FRAMEWORK

a. Universal Declaration of Human Rights (UDHR)

The Universal Declaration of Human Rights (UDHR) is one of the most important international documents shaping discussions on euthanasia, even though it does not directly mention or regulate it. Adopted by the United Nations in 1948, the UDHR sets out fundamental human rights that influence how countries design their national laws on life, death, and medical ethics. In the absence of a specific global law on euthanasia, its principles are often used by both supporters and opponents to justify their positions.

One of the most relevant provisions is Article 3, which states that “*everyone has the right to life, liberty and security of person.*” This article is frequently cited by countries that oppose euthanasia, as it emphasizes the protection of life as a fundamental and inalienable right. From this perspective, intentionally ending a human life—even with consent—can be seen as incompatible with the duty to protect life. As a result, many states interpret Article 3 as a strong legal and moral basis for prohibiting euthanasia.

Another important provision is Article 5, which prohibits “*torture or cruel, inhuman or degrading treatment or punishment.*” Supporters of euthanasia sometimes use this article to argue that forcing individuals to endure extreme, prolonged suffering from terminal illness may be considered a form of inhuman treatment. From this viewpoint, allowing euthanasia could be seen as a way to protect human dignity by preventing unnecessary suffering at the end of life.

The concept of human dignity, which is central to the UDHR’s overall structure, also plays a major role in the euthanasia debate. However, dignity is interpreted in two different ways. Opponents argue that dignity requires preserving life under all circumstances, while supporters argue that dignity includes the right to avoid unbearable pain and to choose a peaceful death. Because the UDHR does not define dignity in a single strict sense, this allows for conflicting interpretations.

The UDHR also emphasizes freedom, equality, and personal autonomy, which are sometimes linked to arguments in favor of euthanasia. Proponents claim that individuals should have control over their own bodies and medical decisions, especially in cases of irreversible suffering. However, the UDHR does not explicitly extend autonomy to include the right to end one’s own life, leaving this interpretation open to debate.

b. European Convention on Human Rights (ECHR)

The European Convention on Human Rights (ECHR) is the key regional legal framework shaping how euthanasia is treated in Europe. Although it does not explicitly mention euthanasia or assisted dying, its articles—especially those protecting life, dignity, and private autonomy—have been widely interpreted in legal debates and court cases involving end-of-life decisions.

The most important provision is Article 2, which protects the right to life. It states that everyone’s right to life shall be protected by law. This article is often the strongest legal argument against euthanasia, as it emphasizes the state’s duty to safeguard life. Many European countries interpret Article 2 as requiring strict protection against intentional killing, including euthanasia. However, the European Court of Human Rights has also clarified that Article 2 does not explicitly require states to criminalize assisted dying in a uniform way.

Another key provision is Article 8, which guarantees the right to respect for private and family life. This article has been central in euthanasia-related cases, as it has been interpreted to include personal autonomy and the ability to make decisions about one's own body and medical treatment. Some individuals have argued under Article 8 that they should have the right to choose the timing and manner of their death. However, the Court has consistently ruled that while Article 8 protects personal autonomy, it does not automatically grant a legal right to euthanasia.

A crucial concept developed by the European Court of Human Rights is the "margin of appreciation." This doctrine allows member states of the Council of Europe to decide how to regulate sensitive moral and ethical issues like euthanasia. Because there is no European-wide consensus, the Court gives countries flexibility to determine their own laws. This is why euthanasia is legal in some countries, such as the Netherlands and Belgium, while it remains strictly prohibited in others.

The ECHR also reflects a balance between state protection of life and individual autonomy. Courts have repeatedly emphasized that protecting vulnerable individuals is a legitimate reason for restricting assisted dying laws. At the same time, they acknowledge that end-of-life decisions involve deeply personal and moral considerations that vary across societies.

c. World Medical Association

The World Medical Association (WMA) is an international organization that represents physicians worldwide and sets global ethical standards in medicine. Although it does not have legal authority over countries, its guidelines strongly influence national medical associations, hospital policies, and doctors' professional conduct. In the debate on euthanasia, the WMA plays a key role because it establishes a clear ethical position on physician involvement in ending life.

The WMA's central stance is that euthanasia and physician-assisted suicide are unethical. It states that a doctor's primary duty is to preserve life, relieve suffering, and provide care, not to intentionally cause death. According to the WMA, deliberately ending a patient's life is incompatible with the fundamental principles of medical practice, regardless of patient consent or suffering.

Instead of euthanasia, the WMA strongly supports palliative care and pain management as ethical alternatives. It emphasizes that patients at the end of life should receive comprehensive care that focuses on reducing physical pain, psychological distress, and emotional suffering. The organization argues that improving access to high-quality palliative care reduces the demand for euthanasia by addressing the root causes of suffering.

A key principle behind the WMA's position is the ethical rule of "do no harm" (non-maleficence). The association argues that intentionally ending a patient's life

contradicts this principle, even if the intention is to relieve suffering. It also highlights the importance of trust in the doctor–patient relationship, stating that allowing euthanasia could undermine patient confidence in medical professionals, especially among vulnerable groups.

The WMA also raises concerns about vulnerable populations, such as elderly people, disabled individuals, or those with mental health conditions. It warns that legalizing euthanasia could increase pressure—direct or indirect—on these groups to choose death, especially in situations involving emotional distress or lack of support. This concern links closely to broader ethical debates about coercion and the protection of human dignity.

Although the WMA maintains a strict opposition, it acknowledges the importance of respecting patient autonomy in medical decisions, such as refusing treatment or discontinuing life support in certain situations. However, it draws a clear distinction between allowing natural death through treatment limitation and actively causing death through euthanasia.

9. LANDMARK STUDIES IN EUTHANASIA

a. Diana Petty Case (United Kingdom)

Diane Pretty, a 43-year-old mother suffering from the terminal stages of Motor Neurone Disease, launched a historic legal battle to secure a "death with dignity." Paralyzed from the neck down and facing a future of increased suffering and loss of speech, she argued that the UK's Suicide Act 1961—which criminalizes assisting a suicide—violated her human rights to self-determination and freedom from degrading treatment. Her specific request was for a "no-prosecution" guarantee for her husband, who was willing to help her end her life. The case traveled all the way to the European Court of Human Rights, which ultimately ruled against her. The court decided that the "right to life" guaranteed by law could not be interpreted as a "right to die," fearing that making exceptions would create a "slippery slope" that might endanger the lives of other vulnerable or elderly people.

b. Terri Schiavo Case (United States)

The case of Terri Schiavo remains one of the most culturally and politically divisive examples of passive euthanasia in American history. After suffering a cardiac arrest that left her in a persistent vegetative state (PVS) in 1990, Schiavo became the subject of a 15-year legal war between her husband, Michael Schiavo, and her parents, the Schindlers. Michael argued that Terri had expressed she would never want to be kept alive by machines, while her parents disputed her medical diagnosis and fought to keep her feeding tube in place. The battle escalated from Florida courts to the U.S. Congress and even involved President George W. Bush. After 14 appeals

and numerous federal interventions, the feeding tube was finally removed in 2005. The case became a global touchstone for the necessity of "living wills" and the ethics of withdrawing lifesustaining treatment from those who cannot speak for themselves.

c. The Chantal Sébire Case (France)

Chantal Sébire was a French teacher and mother who suffered from esthesioneuroblastoma, an extremely rare and incurable tumor that had invaded her sinuses, eye sockets, and brain. The disease caused her horrific physical deformity, constant excruciating pain, and the loss of sight, taste, and smell. In 2008, she made a public appeal to the French courts for the right to physician-assisted suicide, famously stating, "One would not allow an animal to go through what I am enduring." When her request was denied on the grounds that French law did not permit doctors to take active steps to end a life, she was found dead in her home just two days later from an overdose of barbiturates. Her tragic ordeal and the dignity with which she presented her case transformed French public opinion, eventually leading to the Leonetti-Claeys law, which allows for "deep and continuous sedation" for patients near death.

d. The Noah Pothoven Case

Noa Pothoven's case is often cited as a critical example of the complexities surrounding euthanasia for psychiatric suffering. Noa, a 17-year-old girl, had suffered from severe depression, anorexia, and PTSD following childhood sexual assaults. Her death in 2019 led to widespread international misinformation, with many outlets falsely reporting that she had been "granted" state-sponsored euthanasia. In reality, Noa had approached specialized "End-of-Life" clinics, but her requests were deferred or denied because doctors believed she was too young and that further psychiatric treatment should be explored. She eventually died at home after choosing to stop eating and drinking. This case highlights the rigorous "Due Care" criteria in the Netherlands, illustrating that even in countries where euthanasia is legal, requests based on mental health are subject to intense scrutiny to ensure the suffering is truly "unbearable" and that no other medical alternatives exist.

10-QUESTIONS A RESOLUTION SHOULD ADDRESS

1. How can euthanasia be clearly defined to distinguish it from abuse, neglect, or unlawful killing?
2. What mechanisms can be established to monitor and enforce compliance with euthanasia-related safeguards in order to prevent coercion, abuse, and violations of human dignity?
3. How can international cooperation help establish minimum ethical standards while respecting national sovereignty?
4. How should medical professionals be trained to handle ethical dilemmas related to euthanasia requests?
5. What penalties or oversight mechanisms should exist to prevent misuse or unethical practice by medical professionals or institutions?
6. What measures ensure that legalization does not lead to discrimination against vulnerable groups?
7. How can medical professionals accurately assess a patient's mental capacity?
8. What measures can ensure that public awareness and patient education are sufficient for truly informed decision-making?
9. How can access to palliative care and mental health support be improved to reduce the demand for euthanasia?
10. How can NGOs help raise public awareness about patients' rights, informed consent, and available alternatives to euthanasia?

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